# 13}Handling

**Goal:**

**to** enable nursing students to acquire the knowledge, skills, and attitudes necessary for providing safe, evidence-based, and supportive care to women during normal labor.

**Objective:**

-By the end of this session the students will be able to achieve the following:

1) Recognize objectives of nursing care during 2nd stage of labor.
2) Give an example for woman's position during labor.
3) Summarize the preparations that needed for delivery.
4) Estimate when to transfer the woman to delivery room.
5) Assist the women to bear down effectively.
6) Point to the importance of using aseptic technique during labor.
7) Participate in class discussion about protection of perineum and complication of perineal laceration
8) Perform the procedure effectively without assistance.

**Out line:**

1. Objectives of nursing care during 2nd stage
2. When to transfer the woman to the delivery room
3. Woman's position s during labor
4. Preparation for delivery

1. Equipment Procedure

**Introduction:**

## Introduction

The **se**cond stage of labor is the period from complete cervical dilation (10 cm) to the delivery of the baby. It is a critical and physically demanding phase for both the mother and the fetus. During this stage, effective handling by the healthcare team is essential to ensure a safe and positive outcome. Proper management involves continuous maternal and fetal monitoring, supportive care to enhance the mother's comfort and cooperation, and timely interventions to prevent complications such as fetal distress or perineal trauma. The nurse or midwife plays a vital role in guiding the mother's pushing efforts, providing emotional reassurance, and implementing evidence-based practices that promote safe delivery and preserve maternal and newborn well-being.

## Definitions

- **Labor:** Is the process by which the fetus, placenta and membraneare expelled through birth canal.
- **Normal labor:** Spontaneous delivery of full-term, mature, living baby presenting by its vertex through birth canal within reasonable time without interference, without complication to the fetus and mother.

### Identification of 2nd stage of labor:

- ✓ Full dilatation of cervix (this the most sure sign).
- ✓ Desire of patient to evacuate the rectum.
- ✓ Reflex desire to bear down.
- ✓ Bearing down is accompanied by an expiratory grunt.
- ✓ Rupture of membrane.

### Objectives of nursing care during 2nd stage:

- To maintain sterile field during labor
- To maintain health promotion of fetus & mother

- To reduce anxiety & fear of the mother
- To prevent health hazards
- To assist in labor procedure.

**When to transfer the women to delivery room:**

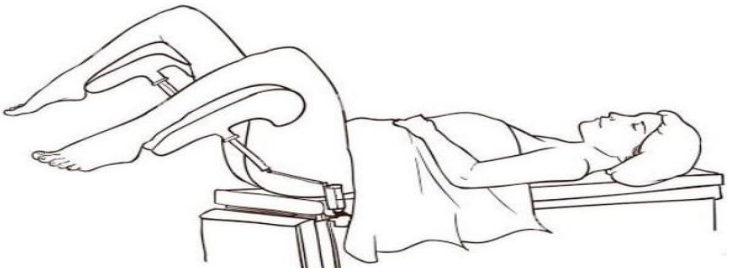
**Prime gravid:**

- Is transferred to delivery room when the cervix is fully dilated and the presenting part is seen (crowning).

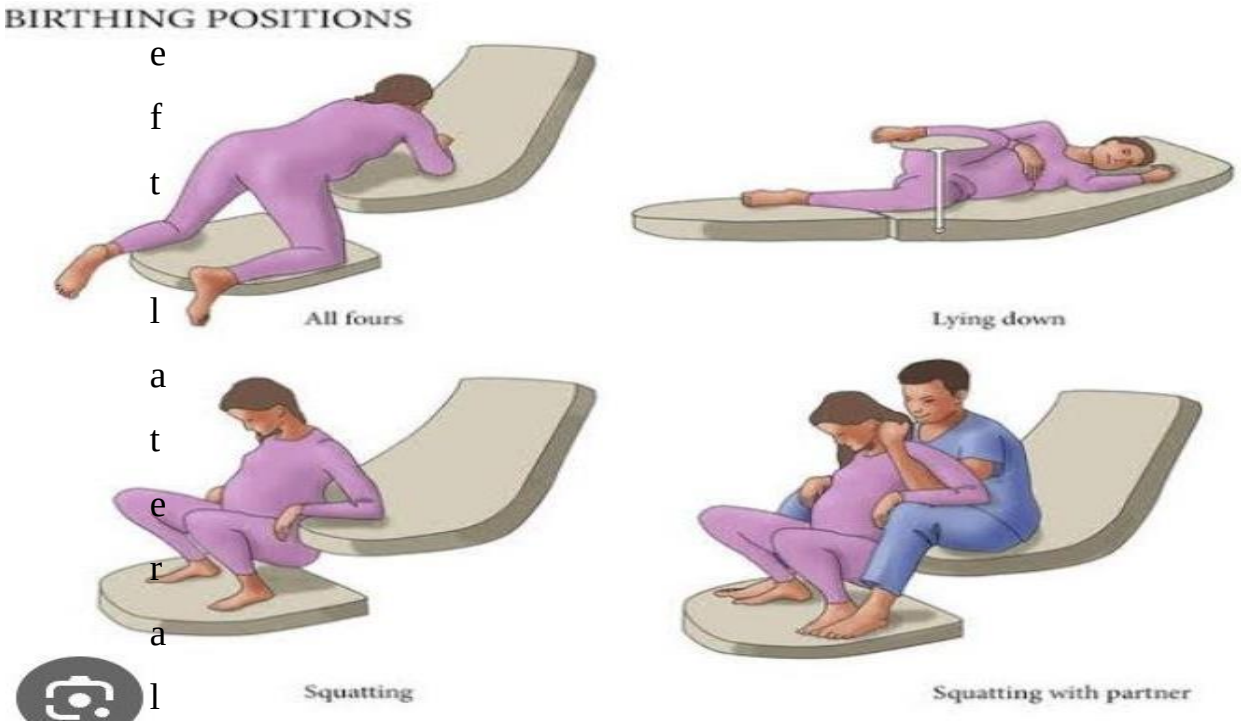
**Multi Para:**

- Is transferred when the cervix is 7-8 cm (3-4 fingers).

**Woman's position s during labor:**



- ✓ Dorsal or lithotomy position
- ✓ Left lateral.




- ✓ Lying down position.
- ✓ All fours position.
- ✓ Squatting & kneeling or standing position

## Preparation:

### 1) Women's preparation:

- Place the women on the delivery table and put her legs on the holders
- The leg and thighs should be dressed with sterile leggings
- Sterile towel should be laid over the abdomen and under the buttocks leaving only vulva and perineum exposed

### 2) Attendant preparation:

- The attendant should put on mask, cap and over shoes
- Hand scrubbing and put sterile gown

### 3) Delivery room preparation:

- Delivery trolley and emergency drugs should be ready
- Delivery room should be warm enough to the infant.
- All equipment should be ready for baby care.
- Enough privacy should be provided
- Strict aseptic technique should be maintained
- Consider woman's safety, she should be transferred between contraction and supported adequately.

## Equipment:

- ☞ Instruments ( 2cokher,2 arteries, 4rings, 1scissor,1needle holder, sem , 1tissue forceps)
- ☞ Sterile drapes - urinary catheter
- ☞ Syringes for local anesthesia, one for methergin
- ☞ Chromic cat gut suture and needle
- ☞ Sterile dressing - 4towels
- ☞ Basin for antiseptic solution

## New born tray:

- ☞ -Cord clamp -identification band
- ☞ -Suction tube - alcohol swab
- ☞ -Scissor, towel - eye drop.

# Handling Guideline

| Action | Rational |
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| 1) Greeting the mother, introduce yourself to her. | -To show interest and initiate good Professional relationship. |
| 2) Explain procedure to the mother. | -To gain cooperation |
| 3) Keeping her privacy (close windows, doors, put curtain). | - To maintain human dignity. |
| 4) Prepare equipment. | - To save time and effort. |
| 5) Hand washing and keep it warm. | - To maintain infection control. |
| 6) Scrubbing, gowning and gloving. | - To prevent spread of infection. |
| 7) Check good place, light and complete equipment. | |
| 8) Put mother in lithotomy position. | |
| 9) Identify signs of starting labor. | |
| 10) Clean perineum from front to back. | To promote cleanliness |
| 11) Place appropriately sterile drapes | To provide sterile field |
| 12) Evacuate the mother's bladder by catheterization. | To promote comfort and enhance uterine contraction |
| 13) Follow up mother progress by P.V. | |
| 14) Instruct the mother to bear down during contraction and relax in between. | To prevent maternal exhaustion and perianal laceration. |

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| **Bearing down.**<br>-Encourage the woman to push during contraction and relax in between<br>-Teach her how to bear down, she take a deep breath hold it, close lips and bears down<br>-The women shouldn't cry or make any sound because much of expulsive force will be wasted | |
| 15) Help the doctor syringe of local anesthesia | |
| 16) call the anesthetists if used any other types | |
| 17)observe presenting part for crowning occur when 3-4 cm of the head appear during uterine contraction | |
| 18)give scissors the doctor to cut episiotomy | |
| 19)support perineum with sterile dressing & good flexion of fetal head at the same time | To prevent perianal trauma |
| 20)once delivery of fetus , clamping the 1st cokher on the cord far from the mother at least 20 cm | |
| 21)evacuate small segment ( Don't do | |

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| milking ) | |
| 22)clamping the 2nd cokher on the cord far from the first about 10 cm | |
| 23)cutting the cord in between down ward using dressing | To prevent splashing of blood |
| 24)assisting in delivery of placenta | |
| 25) Inspection of the placenta by examining fetal& maternal part. | |
| 26) Give syringe of mother gin to circulating nurse to provide it to the mother as advice. | - to detect any deviation from the normal |
| 27) Cover the episiotomy incision with sterile dressing. | |
| 28) Change the towel under the mother. | |
| 29) Assist the doctor in episiotomy repair according procedure. | |
| 30) Ensure the uterus is well contracted. | To avoid post-partum He |
| 31) Make perianal care. | |
| 32) Explore area of episiotomy, swabbing with betadine on the episiotomy site. | |
| 33) Put sterile dressing on the perianal area. | |
| 34) Remove all towels from the mother. | |

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| 35) Collect and clean instrument. | |
| 36) Report complete information about mother & baby. | |

## Handling Checklist

| Action | First Trial | | Second Trial | |
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| | Done | Note Done | Done | Note Done |
| 1- Check good place, light & complete equipment. | | | | |
| 2- Put mother in Lithotomy position. | | | | |
| 3- Identify signs of starting second stage of labor. | | | | |
| 4- Explain procedure to the mother. | | | | |
| 5- Put on mask & overhead. | | | | |
| 6- Scrubbing, gowning and gloving. | | | | |
| 7- Arrange equipment on the tray according sequence of procedure, | | | | |
| 8- Provide perineal care. | | | | |
| 9- Drape the mother. | | | | |
| 10- Evacuate the mother's bladder by catheterization. | | | | |
| 11- Following up mother's progress by p.v. | | | | |
| 12- Instruct mother to bear down | | | | |

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| during contraction & and relax in between. | | | | |
| 13- Give the doctor the syringe of local anesthesia. | | | | |
| 14- Help doctor in providing local anesthesia according procedure. | | | | |
| 15- Observe presenting part for crowning occur when 3-4cm of the head appear during uterine contraction. | | | | |
| 16- Give scissors to the doctor to cut episiotomy. | | | | |
| 17- Assist the doctor to support perineum with sterile dressing & good flexion of the fetal head at the same time. | | | | |
| 18- As the fetal delivered, clamping the 1st cokher on the cord far from the mother at least 20cm. | | | | |
| 29- Evacuate small segment (not make milking). | | | | |
| 20- Clamping a 2nd cokher on the cord far from the first about 10cm. | | | | |

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| 21- Cutting cord between 2 cokher down ward using one dressing. | | | | |
| 22- Assist doctor in expulsion of the placenta. | | | | |
| 23- Inspection of the placenta by examining fetal &maternal part. | | | | |
| 24- Give syringe of methergin to circulating nurse to provide to provide to the mother as doctor advice. | | | | |
| 25- Cover the episiotomy incision with sterile dressing. | | | | |
| 26- Provide perineal care. | | | | |
| 27- Change the towel under the mother. | | | | |
| 28- Assist the doctor in episiotomy repair. | | | | |
| 29- Ensure the uterus is well contracted. | | | | |
| 30-Make perineal care. | | | | |
| 31-Explore area of episiotomy, swabbing with betadine on the episiotomy site. | | | | |
| 32- Put a sterile dressing on the | | | | |

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| perineal area. | | | | |
| 33- Remove all towels from the mother. | | | | |
| 34-Collect equipment & clean instrument. | | | | |
| 35- Prepare the delivery setting to new labor. | | | | |
| 36- Report complete information about mother & baby. | | | | |
| **Total score:** | | | | |
| **Date:** | | | | |

**Student name:** **Instructor name:**